

What Is Private Equity and Why Should Its Increasing Presence in Ophthalmology Matter?

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Private equity (PE) acquisitions in health care have soared in the past decade with an estimated 1,013 health care related service deals closing in 2021 and 863 transactions in 2022.¹ In particular, PE investments have targeted nursing homes, hospital systems, and physician-owned practices across several specialties, including ophthalmology, dermatology, and gastroenterology.² PE investment typically describes a model in which a firm takes majority ownership in a business with the goal of increasing shareholder value through active changes to the underlying business. A PE buyer is a uniquely active financial buyer, which differs from passive investing (eg, investment banking) or playing the role of a strategic buyer (eg, health care organization expanding their business). With respect to acquisition of physician-owned practices, a common PE strategy is to acquire a large physician practice or “platform” practice and then acquire smaller practices within the same specialty to create economies of scale, add managerial expertise, increase ancillary services, and leverage bargaining power with purchasers and payors. Perspectives and current trends in private equity investment in ophthalmology were well described in a recent series of publications that address the potential mismatch in PE fiduciary duty relative to physician principles of beneficence.³

PE-backed acquisitions of ophthalmology and optometry practices increased substantially between 2012 and 2019, with a median holding period of 3.5

years.⁴ There are certainly potential benefits of PE ownership for practicing retina specialists. Theoretically, physicians are freed from business-related issues, allowing more time to focus on patient care. PE firms’ financial expertise can be useful to focus on generating revenue, profit improvement, and financial growth. Consolidated groups likely have greater leverage with suppliers and insurers, allowing them to optimize costs and reimbursements in ways that individuals or small groups may be incapable of achieving. While acquisition has been associated with increases in allowed amount and charges per claim as well as patient volume changes,² optimization of billing and coding may aid in maximizing profits. Furthermore, physician owners receive an upfront cash buy-out at higher multiples of earnings before interest, taxes, depreciation, and amortization (EBITDA) than would be feasible with a partner-to-associate transaction. Conversely, junior associates and future partners typically do not benefit from the initial cash buyout and lose any future opportunities for practice ownership. As this can create skepticism among new recruits and disincentives for associates, PE-owned practices are forced to increase salaries or equity opportunities to recruit talent and maintain associates within their practices. This may be appealing to young retina physicians interested in salaried positions without the stress of practice-building; however, there is considerable long-term impact to salary potential that may not be fully realized yet.

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Despite these potential benefits, there are mixed and conflicting incentives of involved parties. While physicians are motivated to improve patient care through enhanced patient outcomes and reducing adverse events, PE may be motivated to improve cash flow through increased revenue or decreased cost. There is only one cross-section, decreased cost with stable patient outcomes, which will increase value to both the patient and the health care system. Likewise, vitreoretinal surgeons entering practice may seek to optimize remuneration and patient outcomes, whereas PE may be motivated to minimize remuneration to employees as much as possible while allowing for sufficient retention and recruitment. Given that private firms are not required to disclose information regarding acquisitions or practices, there is a dearth of data to evaluate these potential incentive misalignments.

This is of impact to young retina specialists. First-year vitreoretinal surgery fellows surveyed showed 81.4% of respondents were concerned about the effect of PE acquisitions on career opportunities, with the most reported concerns including job security and oversight of clinical operations.⁵ Metropolitan areas are now saturated with PE-owned practices, limiting employment opportunities for young retina physicians who may fear a lack of personal and/or professional growth opportunities within their practices. Opportunities for leadership and growth are needed to maintain a talented workforce. There is a growing cohort of early-career retina specialists with advanced training such as MBA or MPH degrees, who may now lack opportunities to actuate innovative changes to address patient care, population health, financial access, and other issues in their communities. Early-career retina attendings may intentionally join non-PE-owned practices with established partner tracks, which may suddenly evaporate if the leadership decides to sell the practice before partnership is attained. These fears (or realities) may be specifically troublesome to young retina specialists whose decisions are motivated by long-term career horizons, the need to establish a patient base, medical and other student debts, and a complex declining reimbursement climate.

The nature of PE itself has implications for health care, where outcomes depend on long-term sustainability and quality improvements. While the potential impact on patients is difficult to fully assess in the short term, there is concern for negative implications. For instance, PE ownership of nursing homes was associated with increases in short-term mortality and adverse health outcomes for some patients.⁶ Data is lacking on patient outcomes or quality for ophthalmology acquisitions.

Attempts to study PE influence using Medicare data with difference-in-difference methodology have shown limited or modest findings.^{7,8} Braun et al. found small though statistically significant association of PE-acquisition with increases in ranibizumab injections and decreases in bevacizumab injections,⁷ while Singh et al. found increases in aflibercept usage among PE-acquired retina practices.⁸ These studies are limited by the short timeline of practices under PE ownership and by the lack of public data from PE firms and private practices, making the shrouded reality a source of consternation for patients, physicians, and health systems. As senior partners retire and PE practices change ownership, original practice culture and practice patterns may experience more shift from historical patterns to potentially more profit-driven practices.

Finally, concerns of young retina specialists are further complicated by the evolving landscape. For example, the recent Federal Trade Commission rule banning noncompete clauses⁹ may benefit geographically constricted retina specialists and allow greater freedom from leaving large PE conglomerates. But without sufficient protections, those entering practice face significant informational asymmetries that favor PE firms. Monitoring ownership and consolidation of practices is crucial, as it can have long-term effects on pricing, spending, and competition. Future PE deals may one day require direct oversight from policymakers, as the ramifications of PE acquisitions on global health care outcomes and spending become more apparent with greater transparency and further research. There is a critical and ongoing need for additional data, analysis, and attention to this issue to protect patients and physicians alike.

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